# Recommended, reimbursed, and still not completed.

A partnership brief on preventive care that is recommended and reimbursed but not finished, in the safety-net settings where the gap is widest.

## THE PITCH IN 30 SECONDS

PROPOSED · IN DEVELOPMENT

| | |
|---|---|
| PROBLEM | U.S. colorectal-cancer screening sits at **63.5% against a 72.8% target**, and 44.89% in Federally Qualified Health Centers (CY2025 UDS; Michigan 48.74%). After a positive stool test, 24 to 75% of people get the colonoscopy. The services work; **completion** is what fails. |
| SOLUTION | A proposed **weekly routine**, run by the site's own staff: find who is overdue, rank by three transparent rules, remove friction, and follow every positive test to its next step, tracked on a weekly list. No predictive model and no EHR integration in the first pilot; the nightly coded file never leaves the clinic's systems. |
| EVIDENCE | Every lever is already recommended by the Community Preventive Services Task Force (mailed FIT +16.1pp, reminders +15.3pp, navigation +13.6pp). Real-world gains depend on **how well each step is carried out**: in STOP CRC the same programme ranged from -7 to +18 points across 26 clinics, averaging +3.4. This design works on the carrying out. |
| COST | The host clinic contracts my time as implementation lead from month one: a feasibility rate for months 1–3, the full rate from month four only if it decides to continue, agreed in writing before the start (planning ranges: about $8,000 to $10,000; then about $45,000 to $60,000). HSREP adds no fee on top. The evaluation is funded separately, through an academic partner still to be confirmed. Benchmark, not the pilot's cost: about **$91** per additional person screened by mailed FIT (Pignone et al., JGIM 2021, 2019 dollars). |
| THE ASK | One clinic to run a **12-month pilot** after a setup period of up to three months: a **3-month feasibility phase**, then months 4 to 12 only if the feasibility measures are met. Step one is a 30-minute conversation. |

## THE PROBLEM — ADOPTION, NOT SCIENCE

**~8%**
of U.S. adults 35+ receive all 15 high-priority preventive services (Borsky, *Health Affairs* 2018).

**63.5% → 72.8%**
CRC screening, 2023 baseline vs. the Healthy People 2030 target.

**44.89%**
CRC screening in Federally Qualified Health Centers, CY2025 UDS (Michigan 48.74%); the gap concentrates here.

## WHICH ONE ARE YOU? — DIFFERENT PARTNERS, DIFFERENT VALUE, SAME ENGINE

HEALTH CENTERS / FQHCS

### Improve a measure you already report

Host the pilot: raise a UDS/HEDIS measure you already report, inside your existing staffing (about 5 to 8 outreach hours a week). You contract my time from month one and decide at month three whether to go on. Your clinic is named only if it chooses to be.

FUNDERS / PARTNERS

### Fund a pilot with a published cost benchmark

Fund either side: a grant to the host clinic for the implementation, or to the academic partner for the evaluation; months 4–12 released only if the feasibility phase shows the routine can run. Criteria agreed before results; a path from one site to a network.

RESEARCHERS

### Co-design a publishable pilot

An independent evaluation: before and after against the site's own trend, with a comparison clinic where possible, reported with RE-AIM. Plan published first; authorship by contribution. Principal investigator: to be confirmed.

## THE PILOT LOOP — THREE CORE PILLARS, AND A THREE-PILLAR ROADMAP

CORE · WHAT THE FIRST SITE RUNS

- Service redesign & friction reduction
- A simple data feed — one nightly coded 19-column file for the pilot, dates as day counts, key kept at the site
- Implementation enablement — training so the site runs and sustains it

ROADMAP · AFTER A VALIDATED PILOT, NOT IN THE FIRST PILOT

- Predictive AI, explainable
- Agentic & conversational outreach
- Algorithmic equity & governance

`Identify → Rank by transparent rules (days overdue, days since a positive test, prior no-shows) → Reduce friction → Communicate (literacy-tailored) → Escalate (community-health-worker outreach) → Measure, incl. diagnostic follow-up after a positive result. No patient messaging is automated in the first pilot.`

## THE DEMONSTRATION

### A proposed 12-month colorectal-cancer completion pilot, with a decision at month three.

One Southeast Michigan health center below the UDS benchmark, with outreach staff and a quality-improvement lead. **Months 1–3, feasibility**: kits sent and returned, positive tests navigated and booked, staff hours, data completeness. **Month 3**: continue, or stop. **Months 4–12**: the full pilot. The site keeps its workflow and its data; I am implementation lead, contracted by the site; an academic partner, still to be confirmed, evaluates against the site's own trend. Results are published whichever way they go.

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An HSREP initiative in development. All designs are illustrative and future-tense; no patient data has been collected. The first pilot runs on transparent rules and existing staff; AI components are a roadmap item. Operational materials will be reviewed by counsel before any launch.

SOURCES — Borsky et al., Health Affairs 2018 · Healthy People 2030 C-07 (2023 baseline) · HRSA Uniform Data System CY2025, national and Michigan, accessed Aug 2026 · CDC Community Guide (median effects) · Coronado et al., STOP CRC, JAMA Internal Medicine 2018 · Pignone et al., JGIM 2021 (mailed-FIT cost).